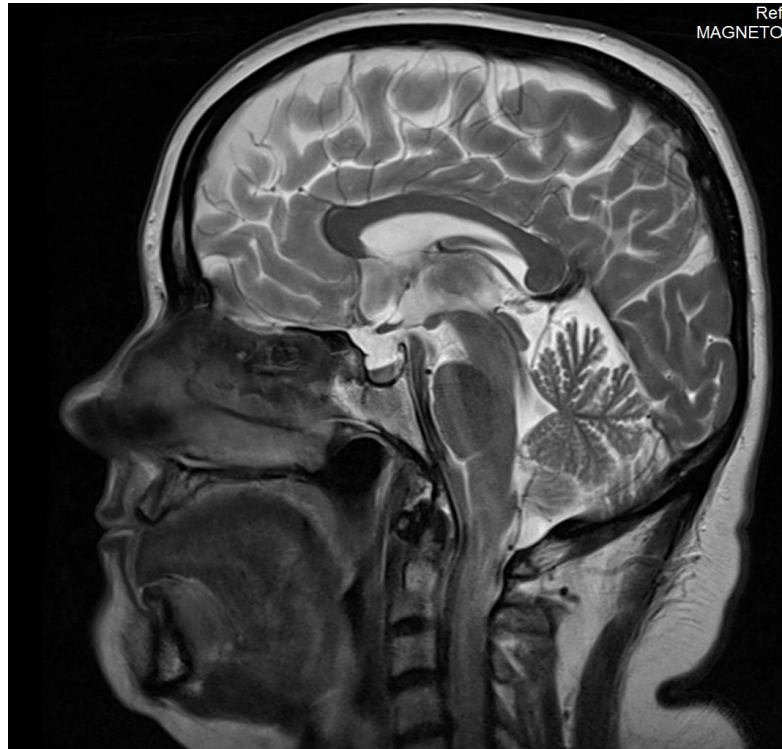


Brain Anatomy & Function



Paul Blyth
Claims Technical and Training

Agenda

Memory Test

What is a benign brain tumour

Signs and symptoms

Treatment options

Risk factors

ABI definitions

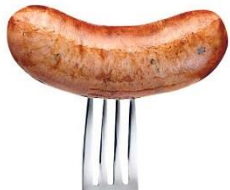
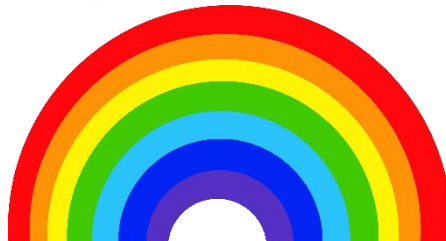
Permanent Neurological Deficit

Claims considerations

Questions



Memory Test



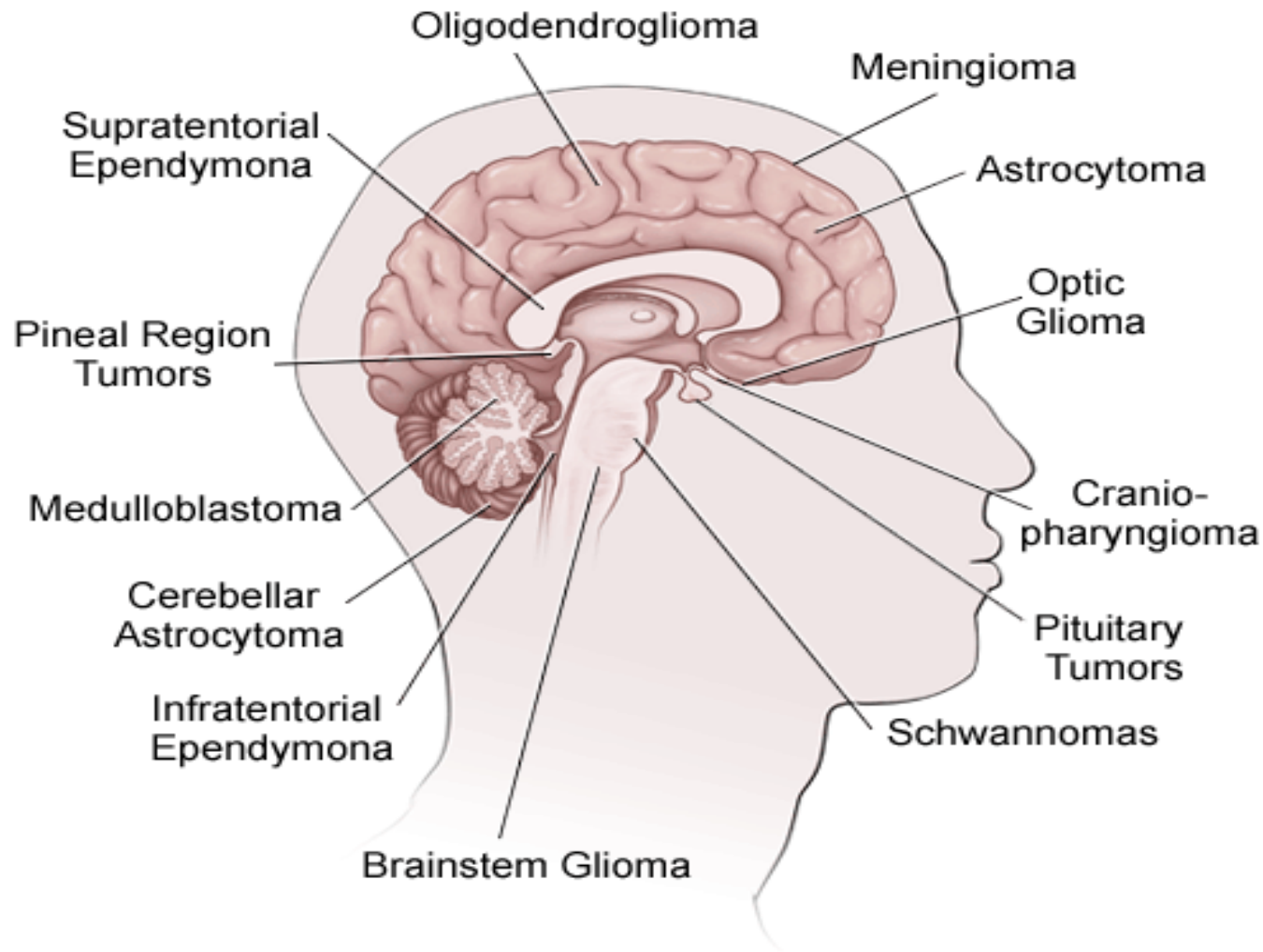
SCOR



Benign Brain Tumour



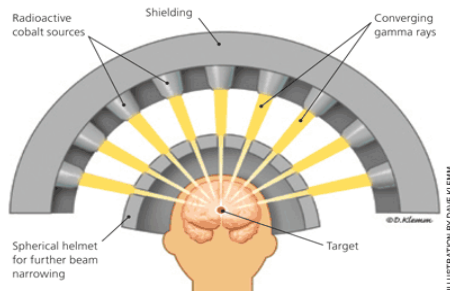
Benign Brain Tumour



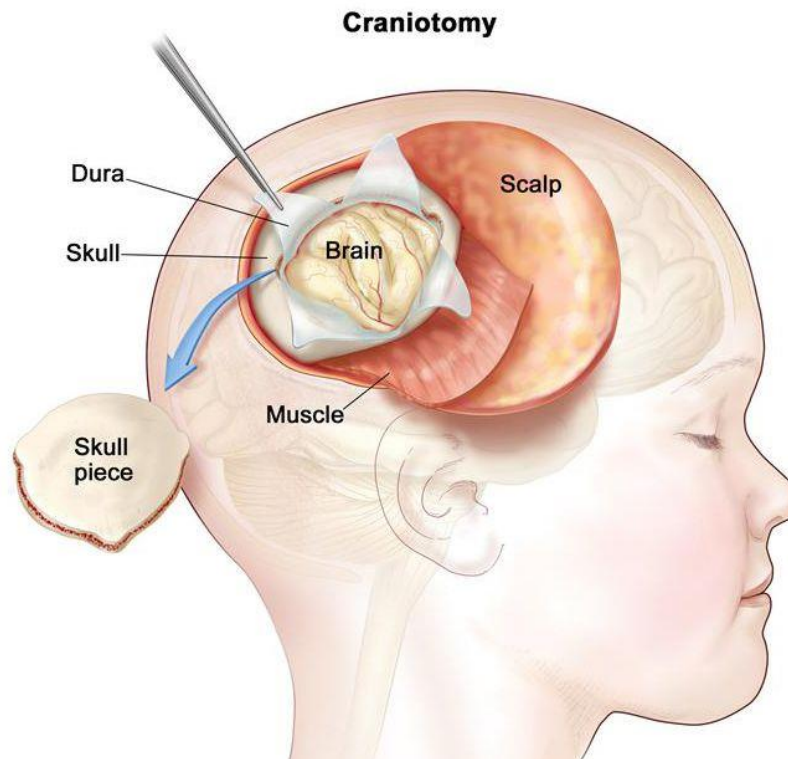
Signs and Symptoms



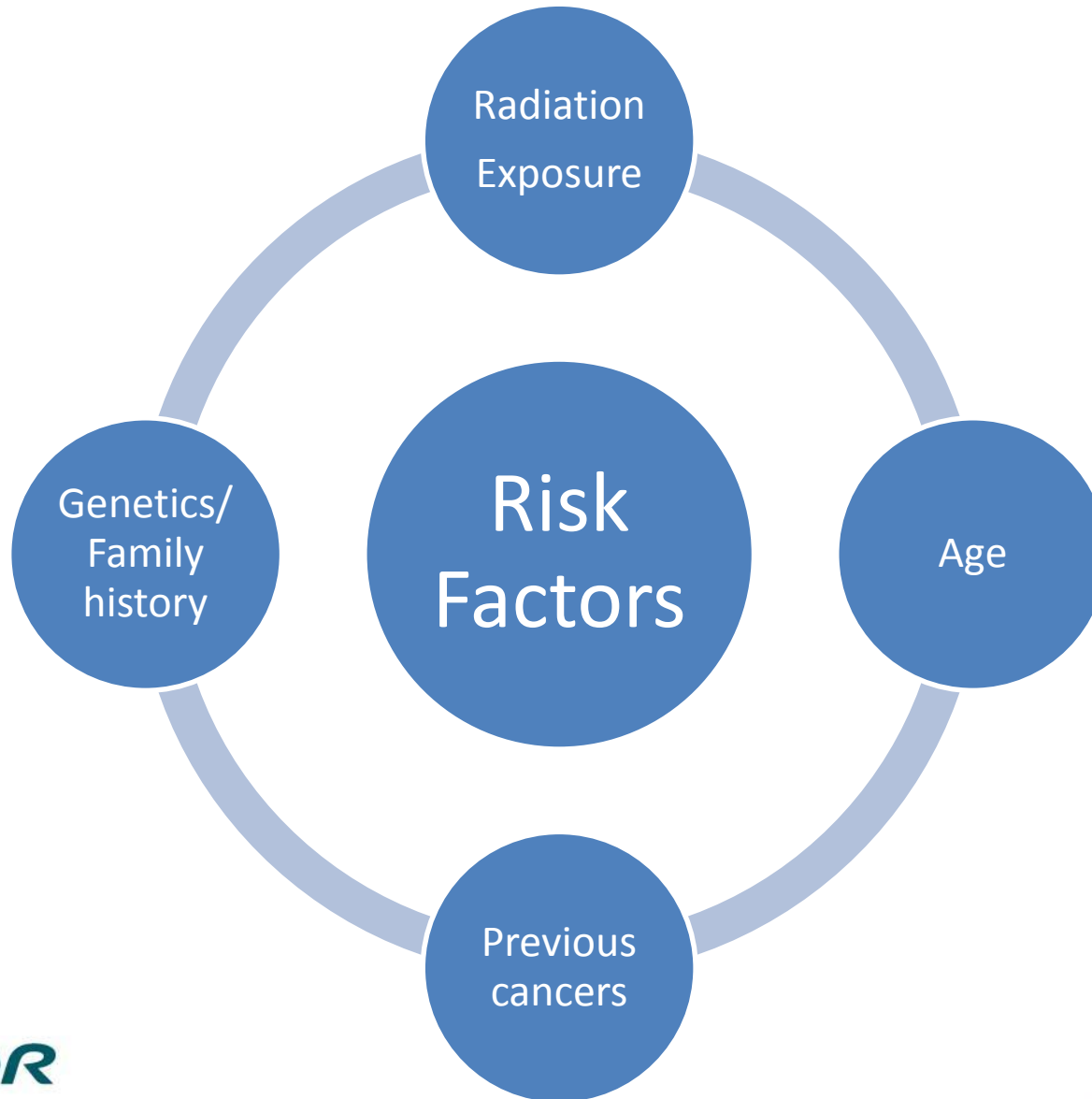
Treatment Options



SCOR



Risk Factors



ABI Definitions

The ABI Statements of Best Practice (SoBP) for Critical Illness Cover May 2002 and August 2004 : *A non-malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered.*

The April 2006 and February 2011 ABI SoPB: *Benign brain tumour - resulting in permanent symptoms.*

A non-malignant tumour or cyst in the brain, cranial nerves or meninges within the skull, resulting in permanent neurological deficit with persisting clinical symptoms.

For the above definition, the following are not covered:

- Tumours in the pituitary gland.
- Angiomas*

The December 2014 SoBP Benign brain tumour – resulting in permanent symptoms

A non-malignant tumour or cyst **originating from the brain**, cranial nerves or meninges within the skull, resulting in permanent neurological deficit with persisting clinical symptoms.

For the above definition, the following are not covered:

- Tumours in the pituitary gland.
- Tumours originating from bone tissue.
- Angioma and cholesteatoma.

Permanent Neurological Deficit

Permanent neurological deficit with persisting clinical symptoms

Dysfunction in the nervous system that is present on clinical examination and expected to last throughout the insured person's life.

To include numbness, hyperaesthesia (increased sensitivity), paralysis, localised weakness, dysarthria (difficulty with speech), aphasia (inability to speak), dysphagia (difficulty in swallowing), visual impairment, difficulty in walking, lack of coordination, tremor, seizures, dementia, delirium and coma.

The following are not covered:

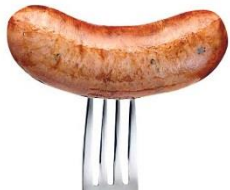
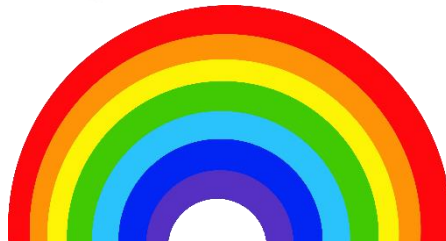
- An abnormality seen on brain or other scans without definite related clinical symptoms
- Neurological signs occurring without symptomatic abnormality, e.g. brisk reflexes without other symptoms
- Symptoms of psychological or psychiatric origin.

Claims Considerations

- Is the tumour operable?
- Is it an excluded tumour?
- Are other treatment options available?
- Will surgery result in full resolution of symptoms?
- Is the claimant only being monitored without further treatment?
- Are the symptoms reported incidental?
 - ***Female patient, diagnosed with a meningioma***
 - ***Presenting symptoms were headache***
 - ***Had a successful craniotomy but headache/migraine persist 2 years later***
 - ***CMO opinion; “The tumour was entirely incidental. Her headaches are caused by migraine - as nearly all headache is.***



Memory Test



SCOR



**Thanks for
listening!**

Any Questions?

**No?
SUPER!**